

GULFSHORE VASCULAR & ENDOVASCULAR

412 Bayshore Boulevard, Suite 300 | Tampa, FL 33606 | (813) 555-0140 | Fax (813) 555-0141
NPI: 1497753102 | Tax ID: 59-5550142 | Fictional practice created for demonstration

PATIENT MEDICAL CHART

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1. Patient demographics

Patient name	Robert Hayes
Date of birth	January 19, 1952
Age at visit	74 years
Sex	Male
MRN	GSV-46002
Address	2501 Bayshore Boulevard, Tampa, FL 33629
Phone	(813) 555-0129
Emergency contact	Patricia Hayes (spouse), (813) 555-0130
Primary care	Dr. Neil Abbott, MD, (813) 555-0155
Preferred language	English

2. Insurance

Primary insurance

Payer	Florida Blue, BlueMedicare PPO
Member ID	FBM229001845
Group	MA-FL-014
Prior authorization	Required for vein procedures
Auth telephone	1-800-555-0133

Secondary insurance. None on file.

3. Active problem list

ICD-10	Diagnosis	Onset	Status
I83.022	Varicose veins of left lower extremity with ulcer of calf	2025	Active
I87.2	Chronic venous insufficiency (peripheral)	2023	Active
I10	Essential hypertension	2008	Active

4. Current medications

Medication	Dose	Frequency	Indication
Amlodipine	10 mg	Daily	Hypertension
Wound care	Foam dressing to left medial ankle	Changed daily	Venous ulcer

5. Allergies

Allergen	Reaction	Severity
Penicillin	Rash	Moderate

6. Visit notes — vascular surgery

Visit date: March 18, 2026 | **Attending:** Elena Vasquez, MD, FACS | **Type:** New patient consult

Chief complaint. Open ulcer above the left medial malleolus for five months.

History of present illness. Robert Hayes is a 74-year-old retired electrician. A wound above the left medial malleolus has been open for five months despite weekly wound clinic care and 20-30 mmHg stockings worn most days since November 2025. The ulcer is venous: it sits in the gaiter area, the leg is swollen, and arterial pulses are normal. He also has aching in the same leg. He has never had a vein procedure. He has no history of deep-vein thrombosis. He wants the veins treated so the ulcer can close.

Examination.

Blood pressure 148/82 mmHg	Ulcer 1.8 cm open ulcer, left medial malleolus, shallow, no bone. Present since October 2025.
CEAP C6, active venous ulcer, symptomatic	Pulses Pedal pulses 2+. Ankle-brachial index 1.05 on the left.
Stockings 20-30 mmHg most days since November 2025. Ulcer has not closed.	

Assessment and plan.

Active venous stasis ulcer, left medial malleolus, CEAP C6.
Order venous duplex to map saphenous reflux and perforators.
Continue dressings and compression while the study is pending. No procedure is ordered today.

Electronically signed: Elena Vasquez, MD, FACS — March 18, 2026 at 8:50 AM

Visit date: April 2, 2026 | **Attending:** Elena Vasquez, MD, FACS | **Type:** Follow-up

Chief complaint. Duplex shows saphenous reflux and an incompetent perforator. He wants both treated on one day.

History of present illness. The duplex confirms left great saphenous reflux and a single incompetent posterior tibial perforator under the ulcer. The great saphenous vein has not been treated or eliminated at any time. The ulcer is still open. He cannot arrange two separate procedure days because his spouse drives him and they live an hour away. He asks for both veins to be closed in one trip to the operating room. The surgeon agrees to schedule them together.

Examination.

Ulcer Still open, 1.6 cm, left medial malleolus	Prior vein surgery None. The saphenous vein has not been eliminated.
Deep veins No history of deep-vein thrombosis	Scheduling One operative session requested

Assessment and plan.

Left great saphenous reflux with an active venous ulcer would, by itself, be an indication to treat the great saphenous vein. The posterior tibial perforator has reflux and measures 4.1 millimeters. It has not been previously isolated by saphenous elimination.
Plan: same-day procedure on April 16, 2026. Endovenous radiofrequency ablation of the left great saphenous vein and endovenous radiofrequency ablation of the left posterior tibial perforator will be performed in the same operative session.

Electronically signed: Elena Vasquez, MD, FACS — April 2, 2026 at 4:10 PM

7. Diagnostic reports

Venous duplex reflux study — March 25, 2026

Ordering physician	Elena Vasquez, MD, FACS
Facility	Gulfshore Vascular Lab
Indication	Nonhealing left medial ankle ulcer
Technique	Upright duplex with perforator survey

Interpretation. Left great saphenous vein: reflux lasting 1.6 seconds. Diameter 7.4 millimeters at the proximal thigh. A posterior tibial perforator 12 cm above the medial malleolus measures 4.1 millimeters and has outward reflux lasting 0.9 seconds. Small saphenous vein is competent. Deep veins are patent and show no chronic post-thrombotic change. Impression: left great saphenous reflux, diameter 7.4 millimeters, and an incompetent posterior tibial perforator, diameter 4.1 millimeters, beneath a medial ankle ulcer. The saphenous vein has not been ablated.

Electronically signed: James Whitfield, MD, RPVI — March 25, 2026

8. Planned procedure

Planned procedure	Same-session ablation of the left great saphenous vein and the left posterior tibial perforator
CPT codes	36475 left great saphenous vein, and 36476 left posterior tibial perforator, same extremity, same session
Date requested	April 16, 2026, one operative session
Prior saphenous elimination	No. The saphenous vein has never been treated.
Surgeon	Elena Vasquez, MD, FACS
Payer	Florida Blue BlueMedicare PPO
Authorization status	Not submitted. Request dated April 2, 2026.

Clinical indication summary

CEAP C6. Active venous stasis ulcer at the left medial malleolus, open since October 2025.

Left great saphenous reflux 1.6 seconds, diameter 7.4 millimeters.

Posterior tibial perforator reflux 0.9 seconds, diameter 4.1 millimeters.

The saphenous vein has not been previously eliminated. Both ablations are ordered for the same operative session on April 16, 2026.

No history of deep-vein thrombosis.

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